

Riverside Endodontics

Jane Doe, DMD, MS
Practice Limited to Endodontics

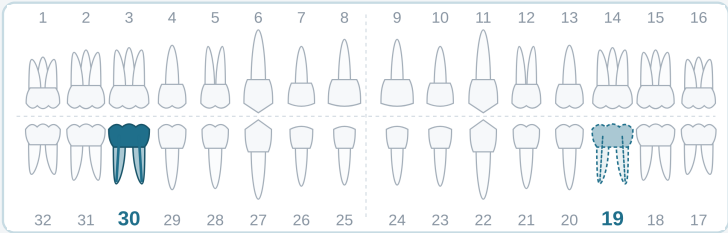
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October 8, 2026

Dr. Amy Lee
Lee Family Dentistry
100 Main Street, Suite 2
Springfield, IL 62701

RE: PATIENT John Smith (ID A-1029) DATE OF BIRTH January 2, 1980
DATE OF TREATMENT October 8, 2026 TEETH #19, #30

TREATMENT AT A GLANCE



2 teeth treated: #19, #30

- Completed
- Initiated

#19 NSRCT - Initiated

Dx Symptomatic irreversible pulpitis / Normal apical tissues
Canals 3: MB, ML, D
Placed Ca(OH)₂ · Temp: Cavit
Your next step Completion visit pending · Defer until endodontic treatment is completed
Prognosis Favorable

#30 NSRCT - Completed

Dx Pulp necrosis / Symptomatic apical periodontitis
Canals 4: MB 21.0, ML 21.5, D 20.5, DL 21.0 mm (WL)
Obturation Warm vertical, bioceramic · Temp: Cavit
Your next step Full-coverage crown within 30 days · Recall 12 mo
Prognosis Favorable

Dear Dr. Lee,

Thank you for referring John Smith to our office. Mr. Smith was seen on October 8, 2026 for evaluation and treatment of teeth #19 and #30. A summary of my findings and treatment follows.

Tooth #19 (mandibular left first molar)

History includes spontaneous pain. Clinical testing revealed lingering pain to cold and no tenderness to percussion and palpation. No swelling or sinus tract was noted.

Diagnosis: Symptomatic irreversible pulpitis with normal apical tissues.

Nonsurgical root canal therapy was initiated under local anesthesia and rubber dam isolation. Three canals (MB, ML, D) were located, cleaned and shaped, and irrigated with sodium hypochlorite and 17% EDTA. Calcium hydroxide was placed as an intracanal medicament. The access was sealed with Cavit. Mr. Smith will return to complete treatment.

Prognosis: Favorable.

Please defer the final restoration until endodontic treatment has been completed.

Tooth #30 (mandibular right first molar)

Mr. Smith presented with sharp pain on chewing for two weeks. Clinical testing revealed no response to cold and electric pulp testing (80/80), tenderness to percussion, no tenderness to palpation, pain on biting, probing depths within normal limits (3 mm or less), and physiologic mobility. No swelling or sinus tract was noted. Radiographic examination: Periapical radiolucency and deep restoration; radiolucency approx. 4 mm at mesial root apex.

Diagnosis: Pulp necrosis with symptomatic apical periodontitis.

Nonsurgical root canal therapy was completed under local anesthesia (4% articaine 1:100,000 epi, 2 carpules, IANB) and rubber dam isolation. Four canals (MB, ML, D, DL) were located, cleaned and shaped, irrigated with 5.25% sodium hypochlorite and 17% EDTA with ultrasonic activation, and obturated using warm vertical condensation with bioceramic sealer. The access was sealed with Cavit.

Of note: Calcified canal(s).

Prognosis: Favorable.

Please place a full-coverage crown within 30 days to protect the tooth against fracture and coronal leakage. We would like to see Mr. Smith for a recall examination at 12 months to evaluate healing.

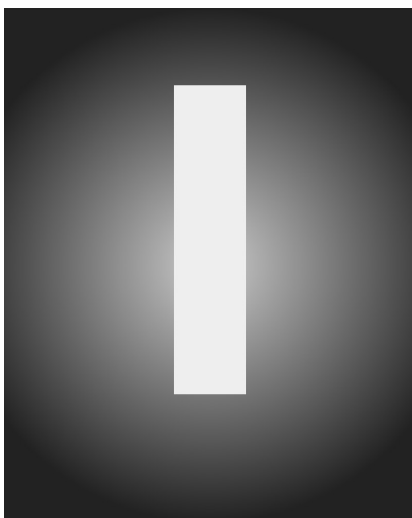
Thank you for the opportunity to participate in Mr. Smith's care. Please do not hesitate to contact me if you have any questions.

Sincerely,

Jane Doe, DMD, MS

Riverside Endodontics

Radiographs



Pre-operative



Post-operative